

An Examination of Care Practices of Pregnant Women Incarcerated in Jail Facilities in the United States

C. M. Kelsey¹ · Nickole Medel² · Carson Mullins² · Danielle Dallaire² · Catherine Forestell²

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Abstract

Objective The number of incarcerated women in the United States has been steadily increasing over the last 30 years. An estimated 6–10% of these women are pregnant at intake. Previous studies on the health needs and care of pregnant incarcerated women have focused mainly on prison settings. Therefore, we examined the pregnancy-related accommodations and health care provided for regional jail populations.

Method The present study is a quantitative survey (administered through phone or email to employees of predominately jail medical facilities) of common practices and policies employed across 53 jail facilities in the United States as a function of geographic region (North vs. South; West vs. Central vs. East). We examined provision of pregnancy screening, special diets, and drug rehabilitation and prohibition of shackling.

Results Strikingly, across all aspects of the care of pregnant incarcerated women there are areas to be improved upon. Notably, only 37.7% of facilities pregnancy test all women upon entry, 45.7% put opioid addicted women through withdrawal protocol, and 56.7% of facilities use restraints on women hours after having a baby.

Conclusion In this first study to examine practices in regional jails nationwide, we found evidence that standards of care guidelines to improve health and well-being of pregnant incarcerated women, set by agencies such as American College of Obstetricians and Gynecologists, are

not being followed in many facilities. Because not following these guidelines could pose major health risks to the mother and developing fetus, better policies, better enforcement of policies, and better common practices are needed to improve the health and welfare of pregnant incarcerated women.

Keywords Incarcerated pregnant women · Health care · Restraints · Shackles · And withdrawal

Significance

What is already known on this subject? Incarceration rates are at all-time highs. Standards of care have been set by agencies such as American College of Obstetricians and Gynecologists for the optimal conditions for the health of the mother and developing child.

What this study adds? Previous literature on the health needs and care of pregnant incarcerated women have mainly focused on women incarcerated in prison settings. This study is the first to nationally examine the national care practices for pregnant women incarcerated in jail facilities.

Introduction

From 1980 to 2010, the number of incarcerated women in the United States increased by 646% (Guerino et al. 2010). In 2013, it was estimated that in county and city jails alone, the average daily population consists of around 97,000 females (Minton 2013). With this dramatic growth in population, there is increased concern for the health care of women, especially for those who are pregnant, in prison

✉ Danielle Dallaire
Dhdall@wm.edu

¹ University of Virginia, Charlottesville, USA

² Department of Psychology, The College of William and Mary, P.O. Box 8795, Williamsburg, VA 23187, USA

and jail systems, which were designed primarily to manage and care for men. Although care practices for pregnant women in the prison system are improving, practices in local jail systems have garnered little attention. This study is the first to examine the accommodations and medical procedures followed by regional jails for pregnant women across the United States.

Incarcerated women generally come from impoverished backgrounds, have minimal education, and have histories of abuse and violence (Luke 2002; Raj et al. 2008). The majority of women in correctional facilities are incarcerated for nonviolent offenses (Raj et al. 2008). Researchers estimate that roughly 6–10% of women are pregnant upon intake (Sabol et al. 2009). However, this may be an underestimation due to the lack of standardized pregnancy testing and reporting across facilities (Clarke and Adashi 2011).

Incarcerated women experience a greater number of health risks than those in the general population (Fisher and Hatton 2009). These health risks, which may or may not have preceded incarceration, include substance use, chronic medical conditions, stress and depressive symptoms, exposure to physical violence, poor nutrition, sexually transmitted diseases, and limited access to reproductive care (Knight and Plugge 2005). The incidence of mental health problems is also higher for incarcerated women (James and Glaze 2006; Lamb and Weinberger 2001). These health disparities can lead to serious pregnancy and delivery complications, such as miscarriage, preterm deliveries, spontaneous abortions, low-birth-weight infants, and pre-eclampsia, placing both the mother and the developing child at risk (Kramer et al. 2000).

Standards of care of pregnant incarcerated women have been set by the American Public Health Association (APHA 2003), the American College of Obstetricians and Gynecologists (ACOG; Committee on Health Care for Underserved Women 2011), and the National Commission on Correctional Health Care (NCCHC 2010). Among many other recommendations, these standards call for prenatal screening tests, special diets, drug rehabilitation when necessary, a prohibition of shackling during labor and delivery, continual access to newborns for mothers after delivery, and breastfeeding support, including breast milk storage. Despite these standards of care for this population, correctional facilities are not required to follow them, so most do not provide adequate care during pregnancy, birth, or the postpartum period (Ferszt and Clarke 2012). According to the Rebecca Project (2010), many state correctional facilities do not require medical examinations as part of prenatal care, nor do they provide prenatal nutrition counseling, special diets, or HIV screening. Very little is known about how many states follow the suggested guidelines, especially in jail facilities.

Methadone maintenance therapy (MMT) is the standard of care for pregnant women with opioid addiction because it protects the fetus from repeated withdrawal episodes, thereby reducing the risk of obstetrical and fetal complications, in utero growth retardation, and neonatal morbidity and mortality (NIH 1998; Kaltenbach et al. 1998). Further, MMT decreases risk for infection for intravenous drug users. In addition, women receive MMT by visiting a clinic and are thereby more engaged with medical professionals. However, because only doctors with special certifications are allowed to prescribe this medication (Benning 2015), many correctional facilities face the challenge of transporting women outside the jail to methadone clinics, making MMT more difficult to attain in a jail setting.

One policy that has received much attention but still poses a significant problem to the health and well-being of pregnant incarcerated women is the use of restraints prior to, during, and after labor. Restraints, or shackles, are defined as any means of physical restraint or devices used to limit range of motion to a person's body, which typically refers to handcuffs, leg shackles, and belly chains (Committee on Health Care for Underserved Women 2011). According to the ACOG, restraints pose significant health risks during delivery and after the birth of the child. These include increased risk of falling and interference with the ability to detect and treat common pregnancy complications (e.g., preterm labor and hemorrhage) during delivery (Kaltenbach et al. 1998). After birth, restraints inhibit the mothers' ability to safely hold her infant and disrupt mother–child bonding.

These findings are further supported by a recent study that surveyed 19 prisons (Ferszt and Clarke 2012). Two of these facilities used belly chains/belts, leg irons, and handcuffs when transporting women to a hospital or clinic, eight used some form of restraints during labor, and six used restraints during the delivery of the baby. Additionally, most did not meet nutritional or other health-related recommendations such as breastfeeding support. Although the Ferszt and Clarke (2012) study helps to provide some insight into the quality of care that incarcerated women receive during pregnancy and delivery, one weakness was the small number of facilities assessed. Moreover, this and many of the aforementioned reports focused solely on prison populations; little research has focused on the care practices of pregnant women in jail facilities (Rebecca Project 2010).

The experience and care provided by prisons and jails may be different in a variety of respects. For example, because prisons typically provide a more stable environment due to the small turnover in residents, care can be established and maintained in the long term. In contrast, women who go to jail are typically incarcerated for a

shorter periods of time, so there is less time to intervene in these facilities.

To address some of these shortcomings, the goal of the present study was to examine jail facilities across the United States to assess common practices and determine the degree to which they adhered to the standards for care of pregnant incarcerated women. Further, exploratory analyses were conducted to determine whether certain jail demographic and health provider characteristics were associated with the standard of care provided.

Methods

Participants

An extensive list of city and county jails (and some prisons, if they also serve as jail facilities, e.g., if they are the only female correctional facility in that state) from across the United States was compiled through searching state corrections pages online. These facilities were all initially contacted by phone to participate in the survey from June 2014 through January 2015.

The survey took an average of 30 min to complete. This study was approved by author's university institutional review board. No compensation was given for participation.

Measures

The survey was developed using questions based on Ferst and Clarke (2012). This was a quantitative survey that

allowed for elaborations or free response to questions pertaining to certain common practices. The questions covered standards of care practices (e.g., guidelines suggested by the ACOG), such as pregnancy screening (e.g., "How are pregnant women identified at your facility?"). Several questions asked about medical care (e.g., "How often do the women get to see the OB/GYN?") and specific medical procedures, such as withdrawal protocols typically used (e.g., "How does your facility treat opioid-addicted pregnant inmates?"). In addition, questions were asked about the accommodations provided to pregnant women, such as fluid and food supplements, furlough and shackling protocols for delivery, and postpartum contact with and ability to provide breast milk to the child. The survey is available from the authors upon request.

Results

We contacted 384 facilities across the United States by telephone. Fifty-three facilities (13.8% response rate) from across 40 states (North=26 vs. South=27; East=18 vs. Central=18 vs. West=17; see Fig. 1 for states included within non-mutually exclusive each category). Table 1 provides demographic information for the jails that responded.

We used a stringent vetting process during the phone calls to obtain a knowledgeable informant. The first attempted contact was always over the phone with the medical department at each jail. However, if no medical staff members were available, we contacted a program officer or other administrator who was familiar with the prenatal

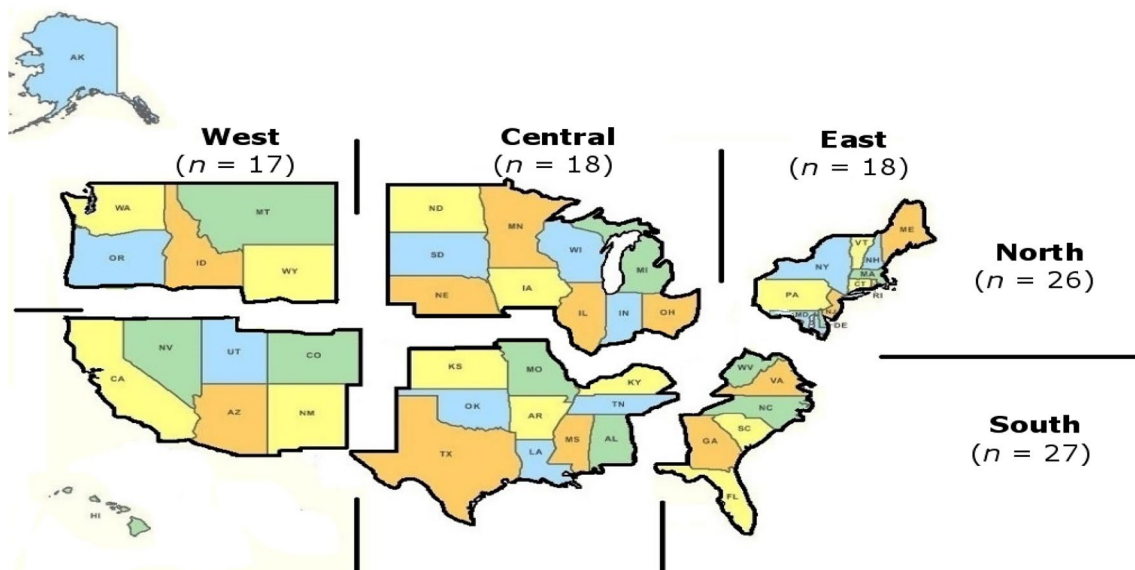


Fig. 1 Distribution of the survey responses by region. Fifty-three jail facilities participated across five non-mutually exclusive regions (*n* values given at the top and side of the figure)

Table 1 Description of participating jails

Characteristic	Percentage / mean (range)
Type of jail	
County jail	89.8%
Regional jail	5.7%
State prison ^a	3.8%
Location of jail	
Rural	54.7%
Urban	34.0%
Suburban	11.3%
Daily census of inmates	578 inmates (range: 6–8389)
Yearly census of pregnant inmates	1–10 pregnant women (range: 1–50+)

^aPrisons were surveyed only if they were the main holding facility for women in that state or area

protocols of the facility. Further, the survey was emailed only in the instance of the designated person not being able to complete the survey at that time. The majority of the jail employees who responded to the survey were medical and health professionals (medical/health=69.8%, mental health=1.9%, other jail administrator=28.3%). Among the health professionals, 39.4% were health administrators, 50.0% were registered nurses and nurse practitioners, and 10.5% were medical directors.

Some jails refused to take the survey because the proper staff members were not available, they were not allowed to discuss their protocols, they did not have a medical department, or an outside company handled their medical services and protocols. Further, some jails did not respond at all to the phone call. Reasons for this include the phone operator not connecting to a staff member or no one answered or returned the phone call. We attribute the low response rate to these refusals and nonresponses. To improve response rates, all facilities were contacted at least twice (with at least one contact by phone and the other was either by phone or email obtained after first phone call). Those who agreed to take the survey were given the option to answer the questions over the phone or to have an online survey sent to them by e-mail. If they chose the online survey (30.2%) and did not respond, a follow-up e-mail was sent within 2 weeks.

Pregnancy Testing and Discussion of Options Available

There was high variability in the common practices followed for pregnant females among jails (Table 2). Fewer than half of the jails reported that they give pregnancy tests to all women who enter the facility (37.7%); most (45.3%) reported that they rely on the woman's self-report of pregnancy status and performed tests to confirm

pregnancy. Upon confirmation of pregnancy, fewer than 30% of the surveyed facilities informed women of options such as adoption or termination. When asked what the jail would do if a women requested a termination of pregnancy, one participant responded, "She would get counseling with a social worker and then would make an appointment with Planned Parenthood. The inmate would pay the cost and make the decision." Another participant responded, "This would be new ground for us to cover." Further, when discussing adoption, one respondent said that the facility "does not discuss adoption during the pregnancy; that discussion happens after the child is born."

Medical Services

All jails reported having onsite medical care, with 41.2% in-house and 58.8% contracted. The majority of the jails reported that they do not charge for OB/GYN medical services (86%). According to survey responses, some states have mandated a \$0 co-pay for all inmate medical expenses. For those that charge for OB/GYN care, the average cost is \$15 per visit (range = \$0–\$35). Almost one-third (31.4%) of the facilities do not have onsite OB/GYN care. Instead, these facilities transport women outside of the jail for medical care. For some of these jails the cost of appointments is dependent on the doctor that the women see. Jails without onsite OB/GYN care indicated that they have to "jump through hoops" to give women proper care. One facility reported that sending women to an outside clinic "hurts convenience, cost, and security."

Only 68% of jails reported providing infectious disease screening. Further, only 44% of facilities reported that ultrasounds are provided to women during pregnancy.

Accommodations

Almost all facilities reported that they provide a bottom bunk, food supplements, and prenatal vitamins (see Table 2). However, it is less likely that a fluid supplement (59.9%) or healthy food options (51%) are provided. One jail employee explained that the food provided is "a pre-packaged meal ... we don't alter it for diabetics, let alone pregnant women."

Nine of the participating jails were asked whether programs were provided for pregnant women in their facility; four (44.4%) said they did not offer any programs for that demographic. Of the five that did, programs included parenting classes and a pregnancy class where women are shown videos on how to raise a healthy baby.

Table 2 Medical care and accommodations provided by correctional facilities, by region (see Fig. 1)

Care/accommodation	North vs. South		East vs. Central vs. West			Total <i>n</i> (%)
	North <i>n</i> (%)	South <i>n</i> (%)	East <i>n</i> (%)	Central <i>n</i> (%)	West <i>n</i> (%)	
Pregnancy tests conducted on all women upon entry	11 (42.3)	9 (33.3)	12 (66.7)	4 (22.2)	4 (23.5)	20 (37.7)
Inform women of pregnancy options						
Right to adopt	8 (34.8)	5 (21.7)	6 (35.3)	4 (23.5)	3 (18.8)	13 (28.3)
Right to termination	7 (30.4)	5 (21.7)	6 (35.3)	3 (17.6)	3 (18.8)	12 (26.1)
Opioid addiction practices for pregnancy						
Withdrawal	10 (41.7)	14 (63.6)	4 (23.5)	8 (53.3)	9 (64.3)	21 (45.7)
Methadone therapy	9 (37.5)	1 (3.7)	7 (41.2)	2 (13.3)	1 (7.1)	10 (21.7)
Accommodations provided						
Bottom bunk	24 (96.0)	27 (100.0)	18 (100.0)	17 (100.0)	16 (94.1)	51 (98.1)
Food supplement	24 (92.3)	24 (88.9)	18 (100.0)	16 (94.1)	14 (82.4)	48 (92.3)
Fluid supplement	11 (44.0)	18 (69.2)	11 (64.7)	9 (52.9)	9 (52.9)	29 (56.9)
Prenatal vitamins	25 (96.2)	25 (96.2)	18 (100.0)	17 (94.4)	15 (93.8)	50 (96.2)
Healthier food options	9 (37.5)	16 (64.0)	9 (52.9)	7 (46.7)	9 (52.9)	25 (51.0)
Infectious disease screen	19 (73.1)	15 (62.5)	12 (70.6)	11 (64.7)	11 (68.8)	34 (68.0)
Delivery						
Granted furlough	11 (47.8)	11 (47.8)	7 (43.8)	7 (46.7)	8 (53.3)	22 (47.8)
Correction's officer allowed at birth ^a	11 (91.7)	10 (100.0)	6 (85.7)	8 (100.0)	7 (100.0)	21 (95.5)
Baby's father allowed at birth ^a	2 (18.2)	0 (0.0)	1 (16.7)	1 (14.3)	0 (0.0)	2 (10.0)
Family member allowed at birth ^a	3 (27.3)	1 (10.0)	1 (16.7)	1 (12.5)	2 (28.6)	4 (19.0)
Birth companion (e.g., doula) allowed at birth ^a	0 (0.0)	1 (10.0)	0 (0.0)	0 (0.0)	1 (14.3)	1 (4.8)
Restraints used ^a						
During labor	4 (33.3)	0 (0.0)	0 (0.0)	3 (37.5)	1 (14.3)	4 (17.4)
After delivery	9 (81.8)	4 (50.0)	2 (25.0)	7 (87.5)	4 (57.1)	13 (56.5)
Postpartum						
Women allowed to provide breast milk for their infant	15 (65.2)	15 (71.4)	9 (50.0)	10 (55.6)	11 (64.7)	30 (68.2)
Women allowed contact visitation with their infant	6 (42.9)	6 (22.2)	5 (38.5)	3 (20.0)	4 (26.7)	12 (27.9)

^aData for facilities that do not grant furlough for delivery, *n* = 24

Substance Use

Almost half of the jails reported that pregnant women addicted to opioids went through withdrawal without the use of symptom alleviators (e.g., suboxone) after entering their facility. One-third reported allowing pregnant inmates to remain on MMT if it was prescribed before they entered the jail, and only 21.7% of facilities reported that they would start MMT if it had not been previously prescribed.

Delivery and Postpartum Contact

Approximately half (47.8%) of the jails reported that they provide women due to deliver while in jail with the option of applying for a furlough, which releases them from jail for their delivery. In situations in which the furlough is not granted, 10% of jails reported that they allow the baby's father to be present at the child's birth. Only 19% allow another family member to be present during birth, and

just one facility would allow a birth companion, such as a doula, to be present during labor. The remaining jails allow only a correctional officer to be with the mother while she delivers.

A sizable proportion of facilities require the women to be handcuffed or shackled during the delivery process (17.4%). This means the women are, as said by one participating facility, "handcuffed by one arm to the bedrail or table during and after delivery." Additionally, more than half of the facilities handcuff or shackle the women post-delivery.

Facilities reported on all of the types of visitation allowed between incarcerated mothers and their newborn children. Thirty-three percent of facilities reported that they allow barrier visits through the glass, 11.1% allow video contact, and 54.2% allow contact visits between mothers and her newborn. Of the facilities that allow mothers to breastfeed (68.2%), the majority (93%) allow women to store the milk. One jail employee described this protocol as

follows: “[They] come to medical to pump, store it there, [and] someone comes to pick it up.”

Demographic Characteristics of Jails that Follow Guidelines

In comparing northern vs. southern states, and western vs. central vs. eastern states (see Fig. 1), jails in eastern states were more likely to conduct pregnancy tests upon entry, $\chi^2(6, n=53)=13.29, p=.04$ (see Table 1). Northern jails, $\chi^2(1, n=31)=8.71, p<.001$, and eastern jails, $\chi^2(2, n=31)=6.26, p=.04$, were significantly more likely to provide MMT for opiate-addicted women. Jails in central states, $\chi^2(2, n=47)=7.28, p=.03$, and northern states, $\chi^2(2, n=47)=8.57, p=.01$, had the highest instance of shackling during the delivery process. In addition, jails in eastern states were least likely to shackle women post-delivery, $\chi^2(2, n=45)=6.41, p=.04$. No other geographic differences were found for accommodations or medical care provided. We found one difference in practices based on number of pregnant women: jails that housed more pregnant women on a given day were significantly more likely to provide MMT than jails that housed fewer pregnant women, $t(27)=-2.36, p=.03$. There were no differences for common practices followed based on the jail setting (rural, suburban, or urban) or on whether medical services were contracted or in-house.

Discussion

The present study reports common practices followed for pregnant incarcerated women in local county and city jails across the United States. The study identifies the quality of care provided to these women, using as a benchmark the standards of care for this specialized population. The high rates of withdrawal for opioid-addicted pregnant inmates and the high rates of shackling and lack of healthful food options are some examples of lack of adherence to the standards of care of pregnant incarcerated women.

Identifying pregnancy is an important first step in providing adequate health care to women who are incarcerated. However, the results of this study suggest that screening for pregnancy is uncommon in jails: only about a third of jails reported this practice. One possible reason for the lower rate of pregnancy screening in jails (37.7%), compared with prisons (68.4%), is that women in jail are sentenced for shorter periods of time than those in prison (Ferszt and Clarke 2012). As a result, pregnant women who go to jail are less likely to deliver in the facility than are those who go to prison. However, given that this group of women is highly transient, this may be the only opportunity they have to receive proper prenatal care. Given the low levels

of pregnancy testing, the rates of pregnant women who are incarcerated may be much higher than previously reported (Clarke and Adashi 2011). Concerted efforts to increase identification in jails throughout the United States, especially in central and western regions, where testing rates are lower, could greatly improve the health and welfare of both mothers and children.

Fewer than a third of surveyed facilities informed women of their pregnancy options, such as adoption or termination. Because there is often only a limited window of time to intervene with pregnant women, it would be beneficial for the health and well-being of the mother and child to have a mandated timeline for jails to follow (e.g., all pregnant women must be seen by a doctor within the first week of incarceration).

Across correctional facilities there were disparities in medical services provided. Almost one-third of the facilities did not have onsite OB/GYN care. Instead, these facilities transported women outside of the jail for medical care. If women are shackled during transportation to the outside medical facility, they may be deterred from going to the medical appointment. Using restraints of any kind during daily routine at the jail puts the mother and developing child at risk by increasing susceptibility to falling. Restraining women during the delivery process interferes with the ability to detect and treat common pregnancy complications (Kaltenbach et al. 1998). Given the health risks posed and increased attention to the issue, it is surprising that the rates of shackling in jail facilities are still so high (e.g., 17.4% during labor and 56.5% after delivery). Since 2000, 21 states have passed laws against shackling pregnant inmates during and after labor, but many of the laws have proved ineffectual (Quinn 2014). In addition to increased enforcement, educating medical providers about their rights to remove any restraint on the patient would help to decrease their use.

The jails did not consistently provide infectious disease screens. This is especially concerning given the heightened rates of blood-borne pathogens in incarcerated populations (Knight et al. 2005). Moreover, provision of MMT to opioid-addicted pregnant women had low compliance among jails (21.7%). Although this practice poses a financial burden to the jail, because often women need to be transported to a methadone clinic for MMT (Benning 2015), the alternative of allowing these women to withdraw poses a health risk to the fetus. Alternatives to withdrawal need to be supported in correctional facilities. Such policies could include licensing correctional care doctors to prescribe methadone, which would decrease the burden of transportation.

In comparing northern versus southern states, and western versus central versus eastern states (see Fig. 1) we found a few geographic trends: eastern states were more likely to conduct pregnancy tests, eastern and northern states were

more likely to provide MMT for opioid-addicted women, and central and northern states were more likely to shackle during delivery. Therefore, efforts to change and enforce protocols for identification, shackling, and MMT may best be concentrated in areas where it is needed. Surprisingly, the setting of the jail (urban, suburban, or rural) did not seem to impact the protocols followed. Future research should continue to investigate patterns of medical care across correctional facilities to identify characteristics that predict better care, to better target interventions to improve quality of and access to care.

One strength of this study is that it is the largest study of medical practices for pregnant incarcerated women. In addition, it provides information about the characteristics of the jails that follow particular protocols. A limitation is the high number of facilities that refused to participate in the study. The jails that did not participate may have refrained because their practices are not perceived as favorable. Moreover, the jails that responded may have been motivated to represent their common practices in an unrealistically positive light. For this reason, the percentages reported here may be an optimistic assessment of the quality of care provided to pregnant incarcerated women. Therefore, it is important to continue research to gain a better understanding of the conditions experienced by this population.

Overall, more needs to be done to improve the health and welfare of pregnant incarcerated women. Although the provision of such care imposes an additional financial burden, improving the well-being and health of pregnant incarcerated women and their unborn children will reduce birth and other longer-term complications that can be costly.

The results of this study demonstrate that in most jails many additional services are needed to meet the standards of care for incarcerated women that have been developed to support healthy pregnancies. This is an important public health goal for correctional facilities, which will significantly reduce health care costs for this underserved population and their children over the long term.

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